NCCP 00371 · VERSION 4

Letrozole · daily hormone therapy

Take letrozole once daily continuously. The prescriber confirms whether the aim is before surgery, after surgery, extended adjuvant treatment or advanced-disease control.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Letrozole: material risks

☐ Joint and muscle pain or stiffness

☐ Vaginal dryness and sexual symptoms

☐ Lipid / cardiovascular risk changes

☐ Hot flushes, sleep disturbance and fatigue

☐ Bone thinning and fractures

☐ Uncommon liver abnormalities or serious allergic reactions

NCCP Child-Pugh C dosing is 2.5 mg every other day; this is not the same hepatic advice as anastrozole. The daily schedule shown is the usual dose and must be checked against the prescription. AIs alone require a confirmed postmenopausal endocrine state. Ovarian suppression is a separate treatment plan and must not be assumed from this card.

Individual benefit, alternatives and concerns

Agreed checks and next review

FBC, renal and liver profile; confirm menopause with hormone tests when uncertain. Assess fracture risk and baseline bone density, particularly for adjuvant AI treatment. The NICE adjuvant baseline-DXA recommendation has an exception for patients already receiving adjuvant bisphosphonates. FBC, renal / liver profile and lipids when clinically indicated. Review bone density during treatment and after treatment according to bone risk. Neoadjuvant response and surgical planning need explicit review milestones.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.2 · Sources checked 2026-10-18
Source reconciled; independent clinical, pharmacy and patient/PPI review pending